



Versiti Wisconsin, Inc
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Client # : 9999
ZZ Test Organization
SAMPLE REPORT
638 North 18 Street
Milwaukee, WI 53233-2121

SAMPLE REPORT

Patient: **Qual, HLA-C**

Birth Date: Gender: Male

Patient ID:

Alt ID:

MRN/Req: 3000600376 - 99990000023047

Physician: Not Provided

Additional Copies:

Histocompatibility

Person Name	Qual, HLA-C
MRN	3000600376
Relationship	Patient
ABO/Rh	
Collected Date	02/01/2019
Received Date	02/01/2019
HLA-C High	C*01:01 C*07:04:01G ^{R1}

R1: HLA-C High
C*07:04:01G = HLA-C*07:04:01G=C*07:04/11

HLA Typing Report

Typing Report Issued:

01-FEB-2019^{I1}

Result Information

I1: HLA Typing Report

Although HLA-C*12 through -C*18 do not have recognized serologic equivalents, they are known to induce specific antibodies.

DPB1 mismatch is classified as permissive or non-permissive based on the relevant Functional Distance (FDAllele) score and associated TCE group classification (Crivello P., et al., Biol Blood Marrow Transplant, 2015; 21(2):233-241).

When only a single antigen or allele is detected, it likely indicates homozygosity and is reported accordingly. However, additional testing would be required for confirmation.

High resolution HLA typing is routinely performed by DNA sequencing using the Sanger chain-termination method (SBT) or next generation sequencing by synthesis chemistry (NGS). HLA NGS panel orders are performed by next generation sequencing, with occasional confirmatory testing by Sanger and/or PCR-rSSO as required due to poor DNA quality or unusual typing results. High resolution HLA typing results meet the American Society for Histocompatibility and Immunogenetics (ASHI) recommendations for resolution of common and well-documented (CWD) alleles. As of January 2015, the high resolution HLA typing results also meet the NMDP requirements for resolving specified CWD null alleles when they exist as a possibility within an assigned G group regardless of the presence or absence of other commonly associated alleles. The version(s) of the IMGT HLA database used to interpret the HLA typing results is available upon request.

Low and intermediate resolution HLA typing is routinely performed by PCR-rSSO or SSP (with melt curve analysis). Some alleles may have been assigned based on their population frequency (Common alleles are reported in lieu of Well Documented or Rare alleles found at very low or unknown population frequencies). When appropriate, intermediate resolution HLA typing is reported with the use of NMDP codes. Allele strings are provided on the report along with each NMDP code. HLA typing results reported with a G or P designation include the indicated unresolved alleles of the G or P group and may also include alleles from unresolved rare-rare allele combinations. When provided, serologic equivalents have been assigned based on the publication "Nomenclature for factors of the HLA system, 2010" by Marsh et al. 2010, Tissue Antigens 75, 291-455, or the publication "The HLA dictionary 2008: a summary of HLA-A, -B, -C, -DRB1/3/4/5, and -DQB1 alleles and their association with serologically defined HLA-A, -B, -C, -DR, and -DQ antigens" by Holdsworth et al. 2009, Tissue Antigens 73, 95-170.

A = Abnormal

C = Critical

L = Low

H = High

CLIA 52D1009037

4499086

Laboratory Director: Matthew W. Anderson, MD, PhD

Printed: 02/01/2019 11:10

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Note: DPB1*04:02 cannot always be resolved from DPB1*105:01 by low resolution typing. However, both are in the DPB1*04:02:01G group, and have identical nucleotide sequences across exon 2 which encodes the peptide binding domain.

This test was developed and its performance characteristics determined by Versiti Wisconsin, Inc. It has not been cleared or approved by the US Food and Drug Administration. This test is used for clinical purposes. It should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments (CLIA) as qualified to perform high complexity clinical laboratory testing.

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